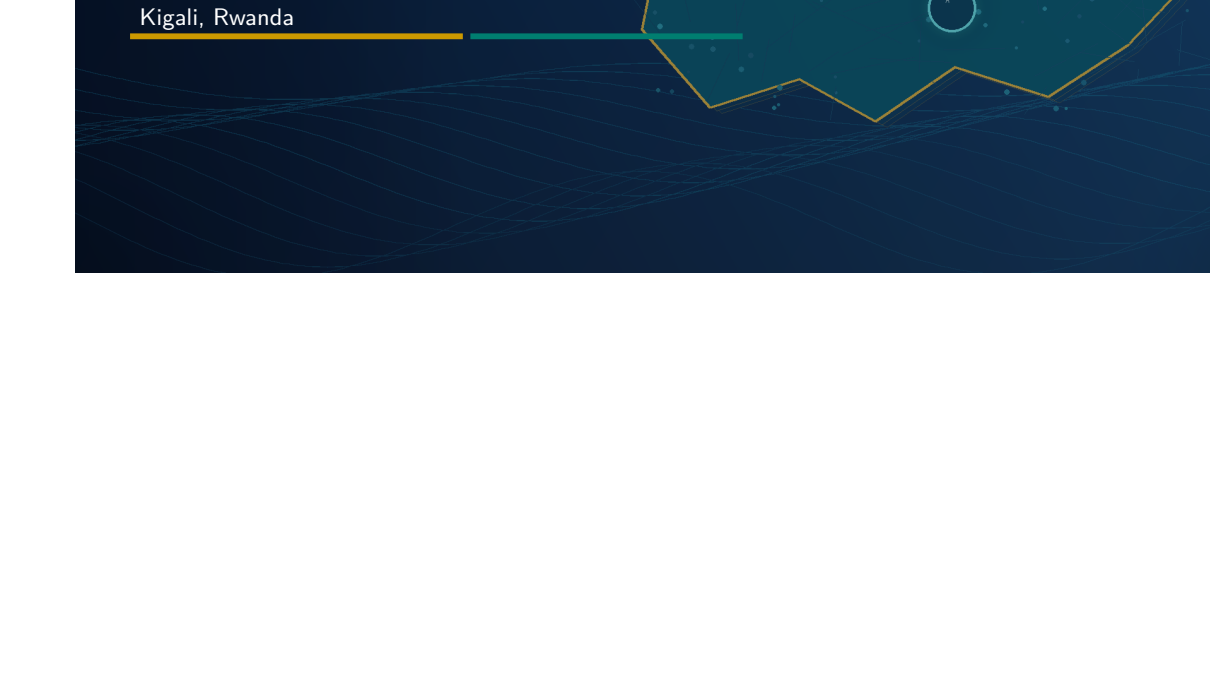


Kigali, Rwanda

The image features a dark blue background with a series of thin, wavy, light blue lines that create a sense of movement and depth. In the upper right corner, there is a stylized mountain range with a jagged, yellow outline. Above the mountains, a small, light blue circle with a white outline is visible, resembling a moon or a star. The overall aesthetic is modern and minimalist.

Background / Motivation

Rwanda-aware public-health motivation

- ▶ HIV transmission is shaped by biology, treatment access, and social behaviour.
- ▶ Awareness, safer behaviour, testing, treatment-seeking, and adherence can change long-term outcomes.
- ▶ In Rwanda, continued prevention and adherence support remain important alongside treatment progress.

Why mathematics? A model gives a controlled way to compare intervention strategies and understand possible long-term behaviour.

Context
HIV in Rwanda

Mechanism
Social behaviour

Method
Fractional memory

Output
Simulation evidence

Problem Statement

Limitations in ordinary modelling

- ▶ Ordinary models usually depend only on the present state.
- ▶ HIV-related behaviour may depend on past awareness, stigma, testing history, and adherence.
- ▶ Social behaviour is often simplified into fixed rates.

Need addressed by this project

- ▶ Include memory using a Caputo fractional derivative.
- ▶ Represent behavioural interventions through effective rates.
- ▶ Use numerical simulation to compare scenarios clearly.

Research focus: How do fractional memory and social behaviour interventions affect simulated HIV dynamics in a SITA framework?

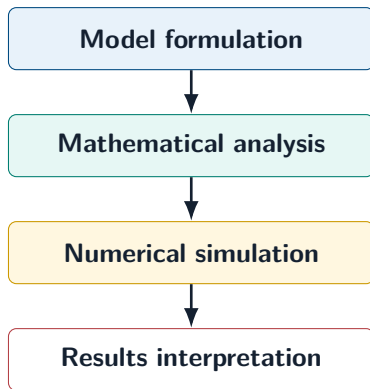
Objectives

Main objective

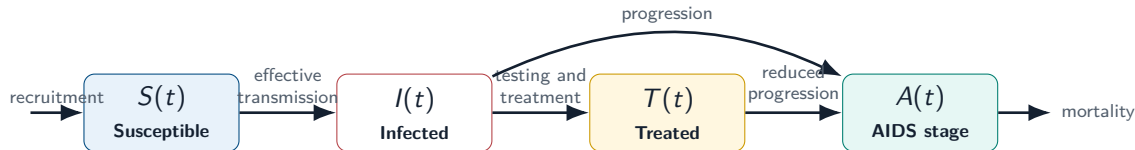
To formulate, analyse, and simulate a fractional-order SITA HIV transmission model incorporating social behaviour interventions.

Specific objectives

1. Formulate the SITA model with Caputo derivative.
2. Derive threshold quantities including $\mathcal{R}_0$.
3. Establish core analytical properties.
4. Implement a numerical simulation scheme.
5. Compare interventions, memory effects, and sensitivity.



Model Diagram: SITA



State variables: $S(t)$ susceptible, $I(t)$ infected, $T(t)$ treated, $A(t)$ AIDS-stage population.

Controls: u_1 awareness, u_2 safer behaviour, u_3 testing, u_4 adherence.

Fractional-Order Model

Caputo memory form

$$\begin{aligned} {}^C D_t^q X(t) &= F(t, X(t)), \quad 0 < q \leq 1 \\ {}^C D_t^q S &= \Lambda - \beta_{\text{eff}} S(I + \eta T) - \mu S, \\ {}^C D_t^q I &= \beta_{\text{eff}} S(I + \eta T) - (\tau_{\text{eff}} + \delta + \mu) I, \\ {}^C D_t^q T &= \tau_{\text{eff}} I - (\rho_{\text{eff}} + \mu) T, \\ {}^C D_t^q A &= \delta I + \rho_{\text{eff}} T - (d + \mu) A. \end{aligned}$$

Intervention-adjusted rates

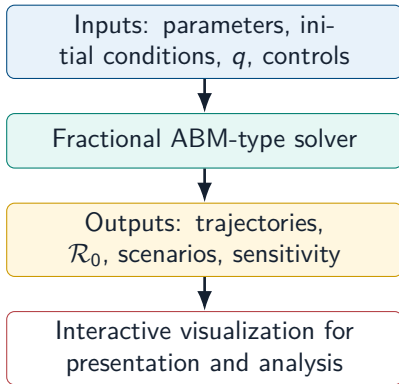
$$\begin{aligned} \beta_{\text{eff}} &= \beta_0(1 - u_1)(1 - u_2) \\ \tau_{\text{eff}} &= \tau(1 + u_3) \\ \rho_{\text{eff}} &= \rho(1 - u_4) \end{aligned}$$

When $q = 1$, the model is ordinary. When $q < 1$, past states influence current change.

Numerical Scheme + Implementation

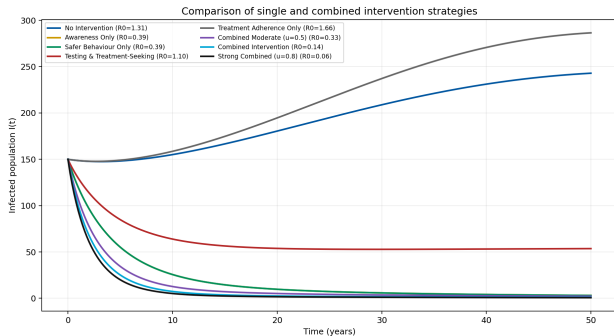
Numerical idea

- ▶ Time interval is divided into discrete steps.
- ▶ A predictor estimates the next state.
- ▶ A corrector updates the state using fractional-memory weights.
- ▶ The same engine is used for baseline, memory, intervention, and sensitivity experiments.



Important distinction: the slides explain the research; the live simulator is used only during the demo to show moving graphs.

Simulation Results: Intervention Comparison

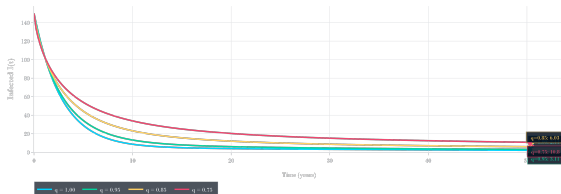


Main interpretation

- ▶ Single interventions help, but each targets only one mechanism.
- ▶ Combined intervention acts on transmission, treatment uptake, and progression.
- ▶ The combined strategy gives stronger simulated control than isolated changes.

Public-health meaning: prevention and treatment support should work together.

Simulation Results: Fractional Memory



Why q matters

- ▶ $q = 1$ represents the ordinary model.
- ▶ Smaller q values strengthen the memory effect.
- ▶ Different q values produce visibly different trajectories.

Defense point: fractional calculus is not decorative; it changes the simulated dynamics.

Live Demo, Conclusion + Recommendations

Live demo plan

1. Open the deployed simulator.
2. Run the moving SITA graph.
3. Compare no intervention with combined intervention.
4. Compare $q = 1$ with $q < 1$.
5. Show scenario and sensitivity views briefly.

Conclusion

- ▶ The SITA HIV model was extended using fractional memory.
- ▶ Behavioural interventions were represented through effective rates.
- ▶ Numerical simulations support the value of combined intervention strategies.
- ▶ Future work should calibrate parameters with Rwanda-specific data.

Final message: this project connects mathematical analysis, numerical simulation, and a working interactive tool for explaining HIV intervention dynamics.